Coastal District Hospital

Discharge Summary

12 Marine Parade, Coogee Heights NSW 2034 | Tel: (02) 9665 7000 | Fax: (02) 9665 7001
South Eastern Sydney Local Health District | NSW Health

Patient Name:	**Nicholas Khan**	Medical Record No.:	**667727**
Date of Birth:	11/06/1954 (Age 71)	Medicare No.:	2271 40415 1
Address:	27 Mill Dr, Bankstown NSW 2200	Next of Kin:	Kyle Khan (Son) 0458 601 716
GP:	Dr Sarah Ng, Eastside General Practice	Allergies:	**Penicillin (rash), NSAIDs (asthma)**

ADMISSION DETAILS

Date Admitted:	05/02/2025 via Emergency Dept	Date Discharged:	10/02/2025
Ward:	8E (Geriatrics)	Length of Stay:	5 days
Admitting Clinician:	Dr H. Vasquez (PGY3 Registrar, General Medicine)	Consultant:	Prof A. Patel (Staff Specialist)
Discharge Destination:	Home (independent)	Discharge Mode:	Self / family transport

PRESENTING COMPLAINT

Mr Khan self-self-presented with a three-day history of pyrexia, productive cough, and right-sided pleuritic pain. CURB-65 score 2 on admission.

PRINCIPAL DIAGNOSIS

Community acquired pneumonia, right lower lobe (ICD-10-AM: J18.1)

ADDITIONAL DIAGNOSES

1. Type 2 diabetes mellitus
2. Hypertension
3. Mild chronic kidney disease (eGFR 65 mL/min/1.73m2)

HOSPITAL COURSE

Admitted with right lower lobe pneumonia (CURB-65 score 2, CORB favouring inpatient management). IV ceftriaxone 1g daily and oral azithromycin 500mg daily commenced as per local CAP guidelines. Sputum culture and blood cultures collected on admission. Blood cultures positive for Streptococcus pneumoniae sensitive to penicillin and ceftriaxone. Antibiotics rationalised to IV benzylpenicillin 1.2g 6-hourly. Clinical settling over 3 days with defervescence and reducing oxygen requirement. Switched to oral amoxicillin to complete a 7-day course on day 4. Follow-up CXR arranged at 6 weeks.

MEDICATIONS AT DISCHARGE

Medication	Dose / Route / Frequency	Notes
Amoxicillin	1g PO TDS	NEW - 4 days remaining to complete 7-day course
Paracetamol	1g PO QID PRN	for pain/fever
Metformin	500mg PO BD	Unchanged
Perindopril	5mg PO mane	Unchanged
Atorvastatin	20mg PO nocte	Unchanged

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KEY INVESTIGATIONS

Bloods on admission (05/02/2025): WCC 18.4 x10^9/L (H), Neutrophils 16.1 x10^9/L (H), CRP 245 mg/L (H), Hb 126 g/L, Creatinine 98 umol/L, Lactate 1.8 mmol/L, Procalcitonin 4.2 ug/L (H).

Bloods at discharge (10/02/2025): WCC 8.2 x10^9/L, CRP 32 mg/L (H).

Imaging: Right lower lobe consolidation with associated air bronchograms. No pleural effusion. No cavitation. Repeat CXR pre-discharge: marked interval improvement of right lower lobe consolidation.

FOLLOW-UP PLAN

1. Re-present if recurrence of fever, worsening shortness of breath, or new pleuritic chest pain.
2. GP review at 1 week. Repeat CXR in 6 weeks to confirm radiographic resolution.
3. Pneumococcal vaccination to be considered if not up to date.
4. Complete the remaining 4 days of oral amoxicillin.

ALERTS AND PRECAUTIONS

ALLERGY: Penicillin (rash)
ALLERGY: NSAIDs (asthma)
Ensure vaccination status reviewed: pneumococcal, influenza, COVID-19.

PREPARED BY

Dr Hamish Vasquez, PGY3 Registrar, General Medicine

Reviewed and approved by: Prof Anika Patel, Staff Specialist, General Medicine

Date: 10/02/2025

Dr H. Vasquez (PGY3 Registrar) Prof A. Patel (Staff Specialist)

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